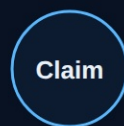


— HEALTHCARE REVENUE INTELLIGENCE & EXECUTION

# AI Revenue Recovery OS

**Predict. Prevent. Work. Resolve. Recover.**

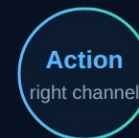
The intelligence and execution layer for autonomous healthcare revenue recovery.



\$ at risk



Understand



Execute

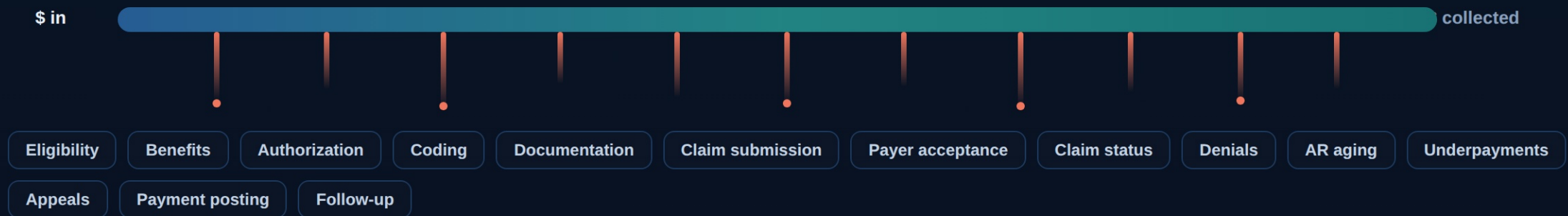


\$ collected

## — THE PROBLEM

# Healthcare revenue is lost in the gaps

Every claim passes through a dozen handoffs between systems, standards and teams. Money leaks at each seam — not because the data is missing, but because intelligence and execution are fragmented across it.



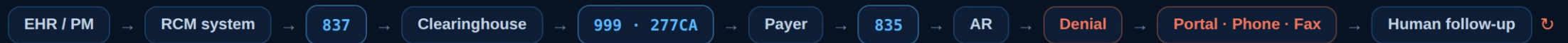
The problem isn't a lack of RCM data. The problem is **fragmented intelligence** and **manual execution** across that data.

## — THE CURRENT RCM MACHINE

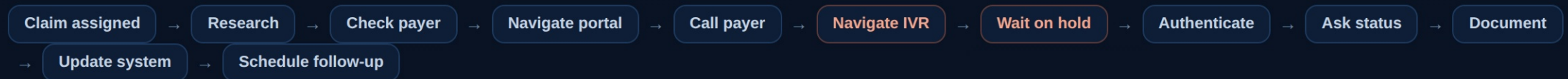
# Disconnected workflows — worked by hand

A single claim traverses a dozen systems and transaction standards before a human ever picks up the phone — then the loop repeats, account by account.

## THE MACHINE — DISCONNECTED SYSTEMS & STANDARDS



## THE HUMAN — HIGH-VALUE EXPERTISE, SPENT ON REPETITION



RCM has become a **collection of disconnected workflows** — not one intelligent system — and expert judgment is consumed working them by hand.

### The reframe

This is **not** "AI replaces AR workers." AI should absorb repetitive execution so human experts focus on **exceptions, judgment and complex recovery**.

## — WHY EXISTING SOFTWARE ISN'T ENOUGH

# Systems of record vs. a system of action

## TRADITIONAL RCM SOFTWARE

- Stores information
- Tracks claims
- Manages workflows
- Provides reports
- Displays dashboards

A passive record of what already happened.

## AI REVENUE RECOVERY OS

- Understands revenue risk & predicts problems
- Prioritizes work & chooses the next action
- Executes the action across channels
- Validates outcomes & learns payer behavior
- Escalates exceptions & measures recovery

An active operator on the revenue cycle.

Traditional RCM software tells teams **what happened**. AI Revenue Recovery OS determines **what should happen next**.



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 WHY NOW

# Four forces are **converging**

For the first time, the data, connectivity and reasoning capability required for autonomous RCM exist at the same time.

**AI**

LLMs + agentic workflows +  
production-grade voice AI

**EDI**

Large-scale machine-readable  
healthcare transactions

**APIs**

Rising payer / provider  
interoperability

**FHIR**

Standardized healthcare data  
exchange

**Labor**

Productivity must rise without  
proportional headcount

For the first time, the **data, connectivity and AI capability** required for autonomous RCM are converging.

## — THE 2026–2027 INTEROPERABILITY INFLECTION

# Payer connectivity is moving toward **API-first** workflows

Under CMS's **Interoperability & Prior Authorization Final Rule (CMS-0057-F, 2024)**, specific CMS-regulated payers face new FHIR API and prior-authorization requirements. This reshapes — but does not fully replace — how revenue workflows connect.

## What the rule requires

- **Prior Authorization API** (FHIR) — end-to-end electronic PA
- **Provider Access API** — claims, encounter & PA data to in-network providers
- **Payer-to-Payer API** — data follows the patient between payers
- **Patient Access API** — enhanced to include PA information

## Who & when

**Impacted payers:** Medicare Advantage, Medicaid & CHIP (FFS + managed care), and QHP issuers on the Federally-Facilitated Exchanges.

### Jan 1, 2026

Operational PA rules: 72 hr expedited / 7 day standard decisions; denial reasons; metrics reporting

### Jan 1, 2027

FHIR API build-out (Prior Auth, Provider Access, Payer-to-Payer, Patient Access)

## LEGACY

Phone → IVR

Human → Notes

## EMERGING

FHIR / API → Structured data

Automated workflow

## REAL-WORLD FUTURE (ALL COEXIST)

EDI

+

API

+

FHIR

+

Portal

+

Voice

+

Human

We orchestrate **all of these channels** — rather than betting on only one.

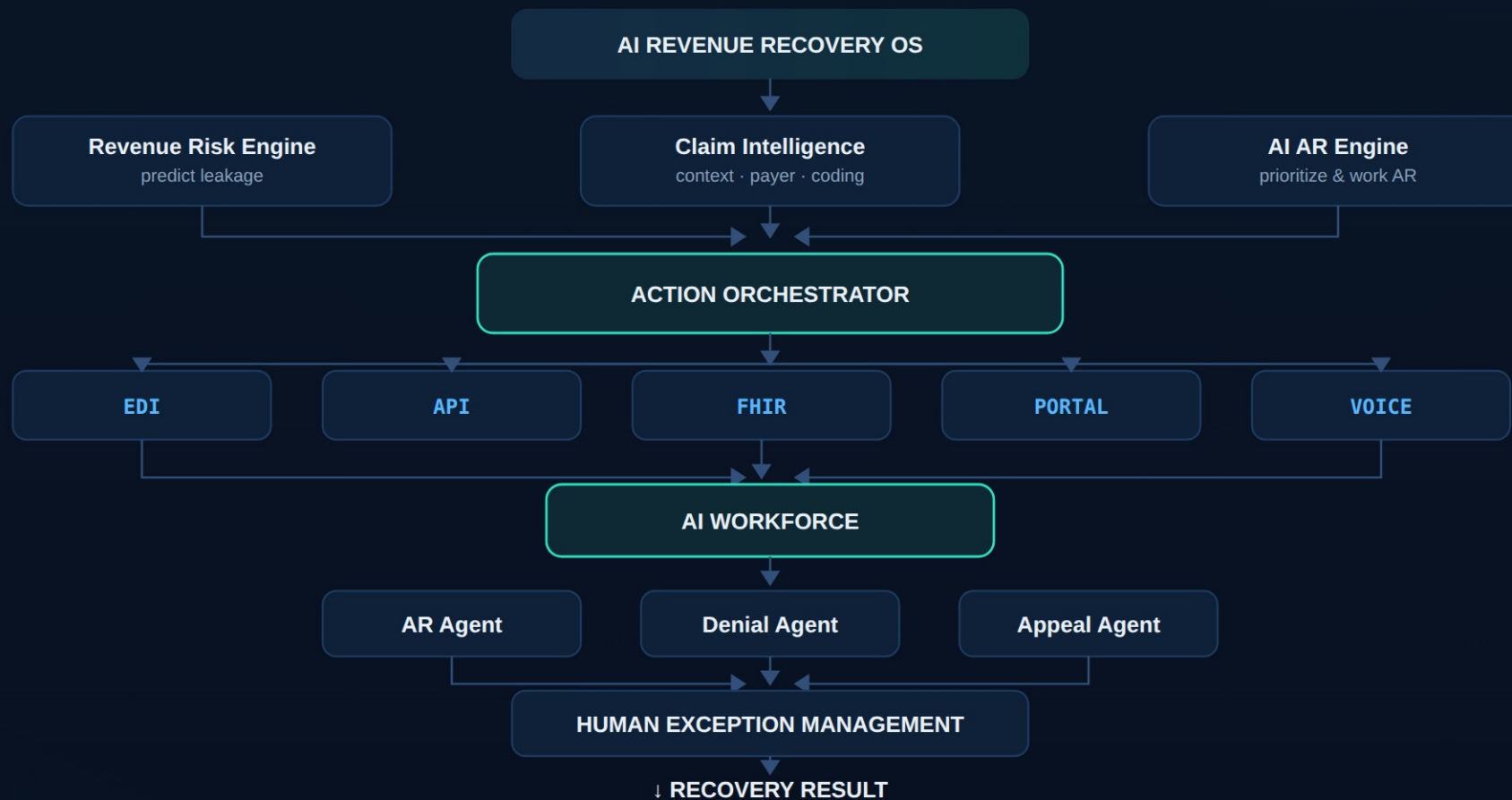
**Accuracy note:** The rule does **not** make all insurance verification API-only, does not eliminate existing X12 transactions (270/271, 278), and does not apply to all commercial or employer plans — only the CMS-regulated payer categories above. Source: CMS-0057-F Fact Sheet & Federal Register (2024-00895), cms.gov.

06

## — THE SOLUTION

# Meet AI Revenue Recovery OS

One intelligence layer reasons over the revenue cycle; one orchestrator executes across every available channel.



One intelligence layer. Multiple execution channels.

## — HOW ONE CLAIM IS WORKED

# A single **\$8,400** claim, worked end-to-end

## THE ACCOUNT

# \$8,400

outstanding balance

## AI ANALYZES

- Payer & claim age
- Procedure & denial history
- Authorization on file
- Timely-filing deadline
- Prior attempts & payment history

### Step 1

Check **EDI** — is status already in the 277 / 835?

### Step 2

Check **API / FHIR** connectivity for the payer.

### Step 3

Check **payer portal** for claim detail.

### Step 4

Decide: is a **call** actually necessary?

### Step 5

**AI Voice Agent** calls payer — only if required.

### Step 6

**Capture:** status, denial reason, payment, reference #, rep, next action.

### Step 7–8

**Validate** response & **update** workflow.

### Step 9–10

**Schedule** next action; **escalate** if confidence is low.

Digital first, voice only when needed, human when it matters — **every step logged and validated.**



## — THE AI AR WORKFORCE

# Turn an AR queue into an **AI workforce**

## HUMAN-ONLY MODEL

- Manual queue prioritization
- Manual research
- Manual calls & portal navigation
- Manual notes
- Manual follow-up

Capacity scales only with headcount.

## AI WORKFORCE MODEL

- Automatic prioritization
- Digital checks first (EDI / API / portal)
- Automated payer workflows; AI calls where required
- Structured documentation & automatic follow-up
- Human exception handling on what matters

Capacity scales with software.

## SPECIALIZED AGENTS IN THE WORKFORCE

AI AR Agent

Claim Status Agent

Payer Calling Agent

Payer Portal Agent

Eligibility Agent

Denial Agent

Appeal Agent

Follow-up Agent

Documentation Agent

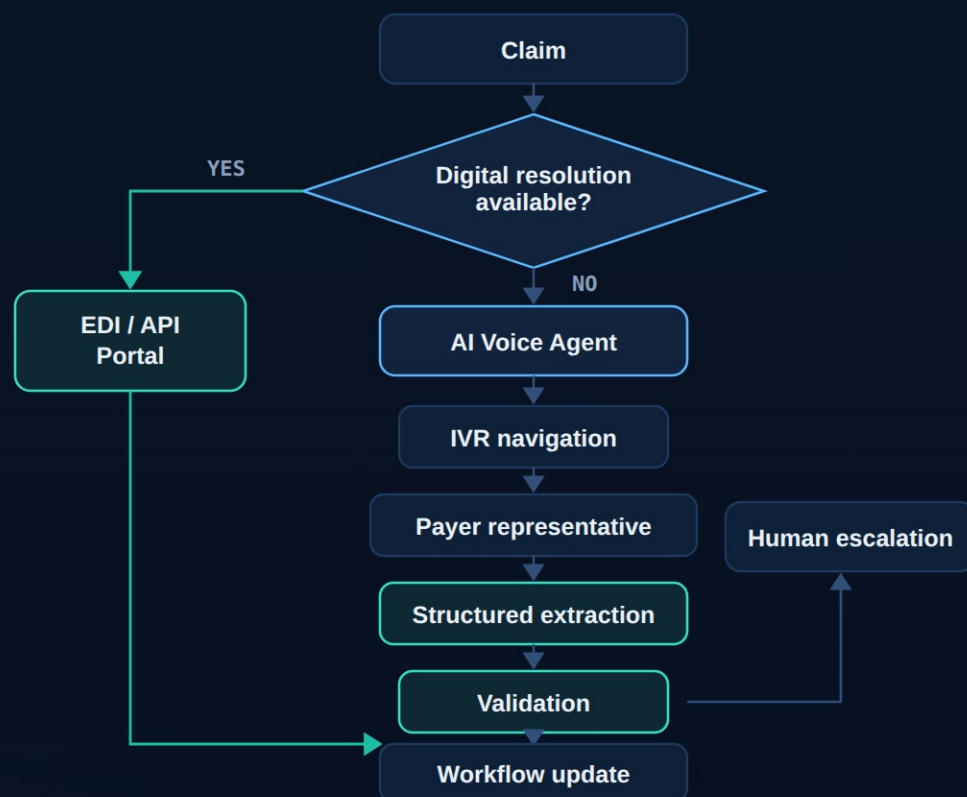
We don't replace the RCM team. **We multiply its capacity.**



## — AI PAYER CALLING

# Voice is a **capability**, not the product

A claim-status request should never automatically become a phone call. Voice is the last resort — invoked only when digital channels can't resolve the issue.



## We name the hard parts

- IVR trees change constantly
- Payer-specific authentication
- Transfers & long hold times
- Inconsistent rep responses
- Payer resistance to poor automation

## How we handle them

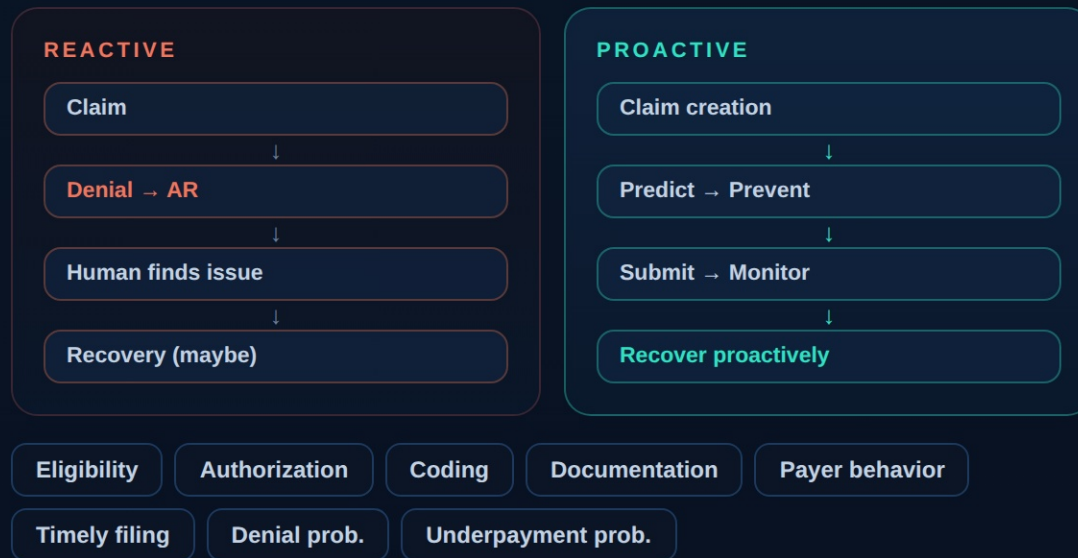
- Payer-specific workflows
- Confidence scoring & evidence capture
- Full audit trails
- Human fallback by design

We do **not** claim 100% autonomous calling. Voice is one orchestrated channel with human fallback — held to accuracy, evidence and audit standards.

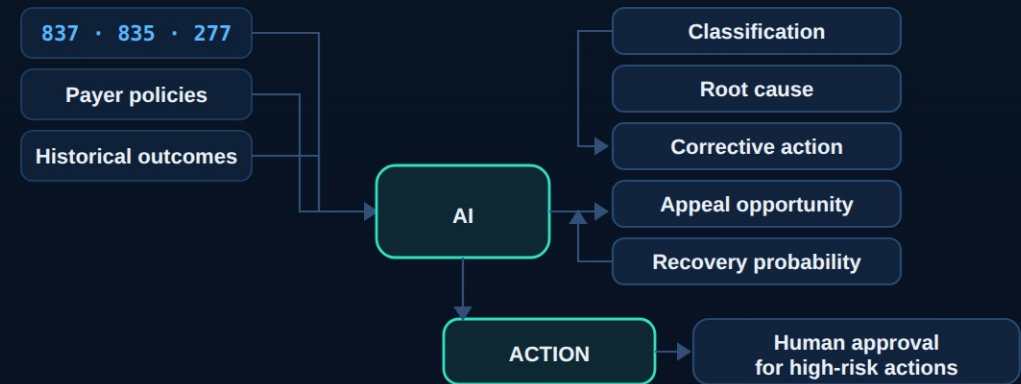
## — REVENUE RISK &amp; DENIAL INTELLIGENCE

# Prevent what you can. Resolve the rest.

## REVENUE RISK ENGINE — PREVENT



## DENIAL INTELLIGENCE — RESOLVE

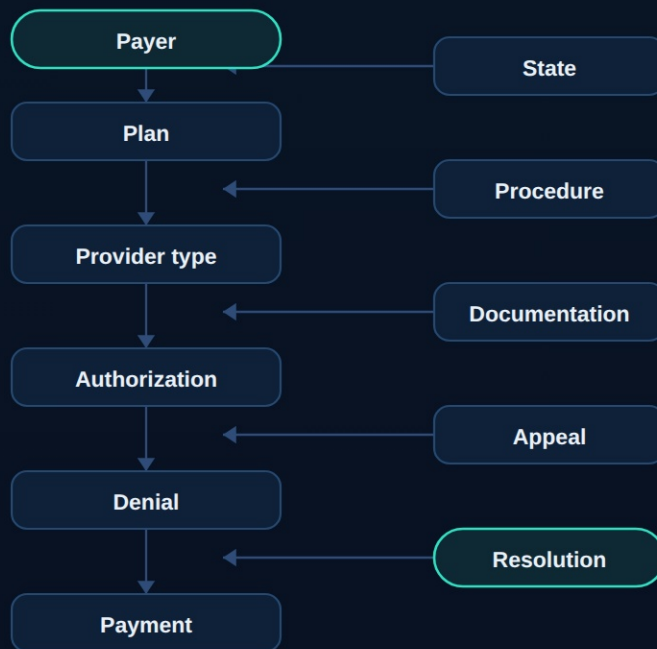


The best dollar to recover is the dollar we **prevent from becoming lost revenue** — and every denial becomes a classified, recoverable action.

Denials rose 20% commercial / 56% Medicare Advantage, 2022 → 2023 (AHA). Rework costs \$25–\$118 / claim (MGMA / Optum). High-risk actions are human-gated.

## — PAYER INTELLIGENCE GRAPH

# Structured operational knowledge of how each payer behaves



## Potential insights

- Payer-specific requirements & denial patterns
- Authorization & documentation behavior
- Successful recovery patterns
- Preferred digital channel & follow-up timing
- Resolution probability

## Privacy & governance first

Intelligence is built from **permitted customer data and workflow outcomes** under strict **tenant isolation**. PHI from one customer is **never** shared with another.

Tenant isolation

Data governance

Permitted learning

Compliance

## — THE PLATFORM LOOP

# A compounding **recovery flywheel**



- **Observe** — claims + EDI + APIs + payer interactions
- **Understand** — AI + rules + payer intelligence
- **Decide** — best next action
- **Execute** — EDI / API / portal / voice / human
- **Validate** — outcome + evidence
- **Recover** — revenue
- **Learn** — improve future decisions

Every resolved claim makes the system **smarter** about future revenue recovery.



## — MARKET OPPORTUNITY

# A large market — sized honestly

RCM estimates diverge ~5–6× because firms define "RCM" differently (services + labor vs. software). We separate the layers rather than picking the biggest number — and AI-in-RCM is the fastest-growing slice.

**TAM**

U.S. RCM market · Grand View 2024

**~\$172B**

~10% CAGR to 2030

**SAM**

RCM software / technology-addressable

**~\$55–60B**

~34% software share

**SOM**

Initial served market —

U.S. RCM / BPO + early provider orgs

billing-outsourcing pool · ~12.6% CAGR

**~\$7B****AI in RCM · ~24% CAGR****BOTTOM-UP RECOVERY WEDGE****3B+ claims / yr · CAQH****11.8% initial denial rate · Optum****50–65% never reworked · MGMA****\$25–\$118 / denial rework · MGMA/Optum****\$20B+ automation opportunity · CAQH**

**Assumptions labeled.** TAM services-inclusive; SAM ≈ software share (~34%), corroborated by Precedence (\$58.5B) & MarketsandMarkets. ACV derived bottom-up in pilots. Sources: Grand View, Precedence, MarketsandMarkets, Market.us, CAQH, Optum, MGMA (2024–25).

## — COMPETITIVE LANDSCAPE

# Everyone owns a slice. No one orchestrates across channels.



## Our differentiation

Cross-channel revenue recovery **orchestration** — selecting and executing the best action across **EDI, API, FHIR, portal, voice and humans**.

## Competitors focus on a slice

- System of record / infra: Waystar, athenahealth, Experian
- Voice automation: Infinitus, SuperDial, Prosper AI
- Autonomous coding: Fathom, CodaMetrix, Nym, AKASA
- Broad RCM AI: Adonis, Candid, QuickIntell
- Payer intel / prior auth: Anomaly, Rivet, Cohere

Positioning reflects each company's public framing as of mid-2026; placement is illustrative, not a claim about specific missing features. Sources: company sites & trade press (FierceHealthcare, MobiHealthNews, HIT Consultant).

## — WHY WE WIN

# Five layers of defensibility

**01****Payer intelligence**

Operational knowledge of  
payer workflows & behavior.

**02****Cross-channel  
orchestration**

EDI + API + FHIR + portal +  
voice + humans.

**03****Workflow intelligence**

Understanding what should  
happen next.

**04****Outcome data**

Recovery outcomes  
improve future decisions.

**05****Integration depth**

Deep RCM-workflow  
integration → switching  
costs.

The moat is **workflow intelligence, payer intelligence, integrations and outcome data** — not the underlying language model.

## — CUSTOMER &amp; BEACHHEAD

# Start where the ROI is measurable

**Primary beachhead****Medical Billing / RCM / BPO Companies**

- Large, repetitive workloads & direct labor economics
- Clear AR pain & measurable ROI
- Operational maturity, KPIs & faster pilots

**Secondary**

Physician groups · specialty practices · hospitals · health systems & ambulatory orgs

**The wedge****Start with AI Claim Status + AR Follow-up**

High volume · repetitive · measurable · human-heavy · clear ROI — and no core RCM-system replacement required.

**THE FIRST PRODUCT DOES EIGHT THINGS WELL**

- 1 · Ingest AR queue
- 2 · Prioritize
- 3 · Check EDI / API / portal
- 4 · Call if required
- 5 · Capture structured result
- 6 · Update workflow
- 7 · Schedule follow-up
- 8 · Escalate

Win the queue everyone hates. Then expand.

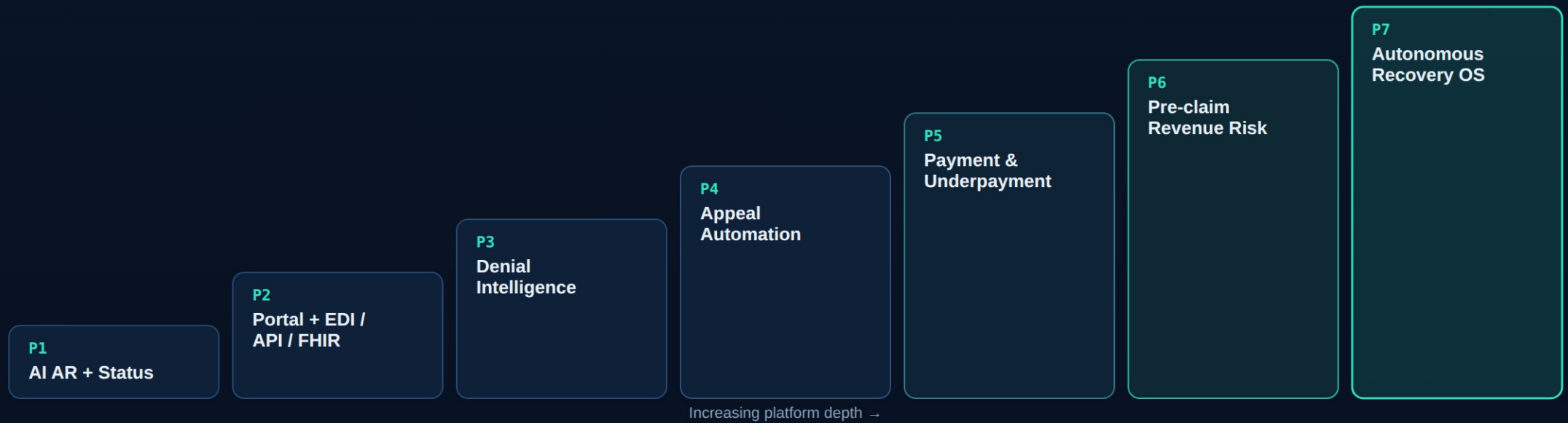
**EXPANSION PATH**

RCM / BPO → Provider orgs → Health systems



## — PRODUCT ROADMAP

# From a workflow to an **operating system**



## — BUSINESS MODEL &amp; ROI

# Priced on **operational value** — proven in ROI

## PRIMARY — PLATFORM + USAGE

**Platform subscription**

Base access, workflow engine, security.

**Per workflow / claim**

Usage-based automation of worked claims.

**AI voice usage**

Metered where voice is actually invoked.

**Enterprise integrations**

Deep EHR / RCM / clearinghouse connectivity.

**Optional — Performance**

Recovery-based pricing for selected workflows — aligning our upside with recovered dollars.

## ILLUSTRATIVE ROI — NOT ACTUAL CUSTOMER RESULTS

**~48%**

automation rate on eligible claims

**2–3×**

effective throughput per FTE

**↓ cost**

lower cost per claim worked

**faster**

reduced days in AR

Assumed inputs: 25 FTEs · 40 claims/FTE/day · ~22k claims/mo · 60% automation-eligible · 20% human intervention.

Claims / day

Cost per claim

Automation %

Intervention %

Resolution time

Denial overturn

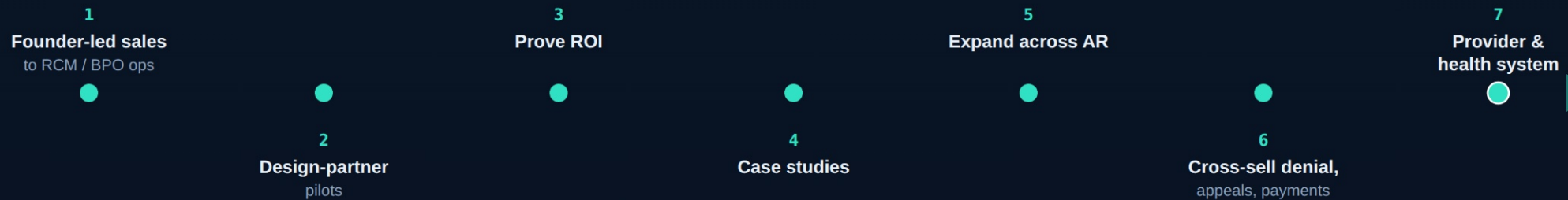
Days in AR

Recovered \$

We monetize against **operational value and recovered revenue** — not AI minutes. Pricing illustrative; validated through pilots.

## — GO-TO-MARKET &amp; PILOT

# Land narrow. Prove it in 90 days.



## THE 90-DAY PILOT — NARROW, MEASURABLE, ROI-VALIDATED

## Days 0–30

Workflow discovery · integration · security review ·  
baseline metrics

## Days 31–60

Controlled AI AR deployment on a scoped queue ·  
human-in-the-loop tuning

## Days 61–90

Scale volume · measure · optimize · ROI validation

## SUCCESS METRICS — AGREED UP FRONT

Automation %

Accuracy %

Human escalation %

Claims processed

Resolution rate

Avg resolution time

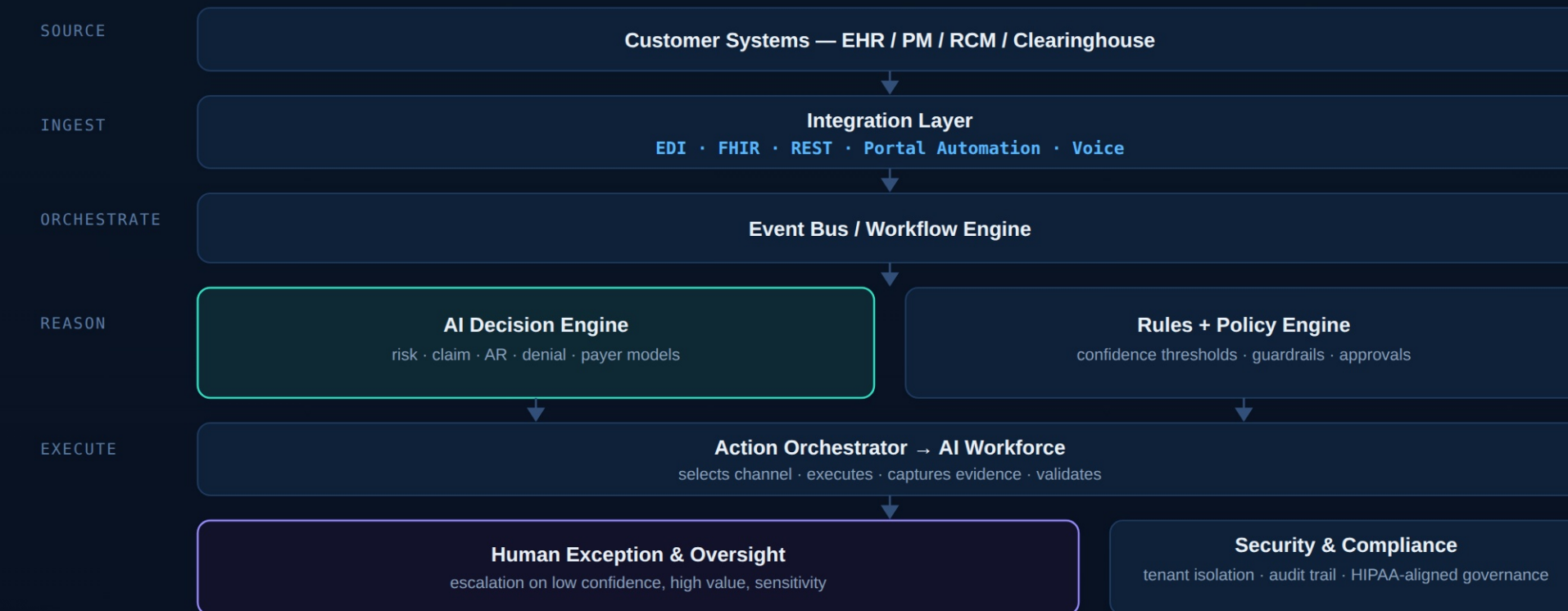
Recovered dollars

Cost savings

20

## — TECHNOLOGY ARCHITECTURE

# Production-grade, governed by design



Every automated action carries a confidence score, an evidence trail and a policy gate — with human oversight and tenant-isolated, HIPAA-aligned data governance throughout.



— THE INTELLIGENCE & EXECUTION LAYER

Existing RCM software manages the revenue cycle.  
AI Revenue Recovery OS **actively works it.**

**Predict. Prevent. Work. Resolve. Recover.**

The intelligence and execution layer for autonomous healthcare revenue recovery.